

AISH vs ADAP: The Language That Decides Your Program

A deep dive on the wording to use — and to avoid — on the Disability Income Assistance application (DS2444A) and the medical report (DS2444B).

Prepared July 2026 · grounded in current Alberta.ca eligibility criteria · general information, not legal or medical advice

The whole thing turns on one verb.

AISH is for a disability that **permanently prevents** employment. ADAP is for a disability that **significantly impedes but does not prevent** employment. ***Prevents*** vs. ***impedes***. Every wording choice on both forms should point at the true one of those two words for your situation.

1. Why the wording decides the outcome

There is now a single combined application for both programs. You do not pick which one you get — an adjudicator reads your file and places you. Understanding how that placement works is the key to understanding why language matters so much.

Here is the mechanic that most people miss: **an adjudicator can grant ADAP on their own. AISH is different — the file has to be referred up to a panel of medical professionals to be determined.**

Alberta.ca describes it plainly: “an adjudicator will review the medical information to decide if an applicant qualifies for ADAP or should be referred to a panel of medical professionals to determine eligibility for AISH.”

The consequence for your wording: moderate, hedged, or ambiguous language settles at the adjudicator’s desk, and the adjudicator’s desk is ADAP. To reach AISH the medical evidence has to be clear and strong enough to make referral to the panel the obvious next step. An unclear file defaults *down*, never up. That is the single most important thing to know before you write a word.

2. The official test, word for word

These are the exact criteria from Alberta.ca as of July 2026. Read them slowly — your forms are effectively answering these two paragraphs.

AISH (medical)	“You must have a medical condition that is likely to remain permanent.” “Your medical condition must be the main factor limiting your ability to earn a living, not other factors such as your education level.” In general, “a severe disability that results in them being permanently unable to work.”
ADAP (medical)	“You must have a severe disability that significantly impedes employment continuously or episodically.” “The severe disability must be determined to significantly restrict but not prevent a person from being employed.” In general, “a severe disability who is assessed as able to work.”

Notice three fault lines, not one. (1) Effect on work: prevents vs. restricts. **(2) Permanence:** permanent and not improvable vs. potentially improvable with treatment/training. **(3) Cause:** the medical condition itself is the main factor, not education, experience, or the job market. Your language has to land on the AISH side of all three, and only where that is clinically and honestly true.

3. The fulcrum vocabulary

Certain words do heavy lifting. The left column signals “permanently prevented from working” (AISH). The right column, even when written by someone with a serious disability, reads to an adjudicator as “able to work with the right support” — which is the definition of ADAP. Use the right-hand words only when ADAP genuinely is the accurate finding.

AISH — what keeps the file here	ADAP — language that routes you here
Prevents, precludes, makes impossible	Impedes, restricts, limits, makes harder
Cannot / unable to sustain work	Struggles with / finds difficult / manages with effort
Permanent; not expected to improve with treatment	Manageable, stable on treatment, improving, responding well
Options tried and exhausted; no therapy will restore work capacity	Currently in treatment; may benefit from further therapy/rehab
Unpredictable incapacity — cannot commit to any schedule	Has good days and bad days
Cannot reliably start, organize, or finish tasks unaided	Needs some help / does better with support
No accommodation makes competitive work reliable or sustained	With accommodation / the right workplace, could work
The disability is the main factor barring work	If retrained / with experience, employment is realistic

The accommodation trap. This is the most common way a strong file gets routed to ADAP. The instinct is to show you’re a willing, reasonable person: “I could work if I had a flexible schedule / could work from home / had the right supports.” That sentence is, almost verbatim, the ADAP test. “Capable with support” is the ADAP finding. If work is genuinely not sustainable for you, the report must say that no accommodation makes it reliable — an employer can adapt a workstation, but none can adapt to unpredictable incapacity.

4. Your part (DS2444A): where wording matters

Most of your form is factual — answer it truthfully and completely. A handful of fields are where language decides direction. The governing principle across all of them: willingness is not the test; the ability to sustain work is. Say you are willing where that's true, and put the “can't” in the fields that ask about ability.

Field	What to write and why
Q13 — Willing to work?	Answer honestly, usually Yes. This asks about willingness, not ability. Saying Yes does not hurt you — willingness is not what the programs test. Do not talk yourself into a “No” you don't mean. Put the <i>can't</i> in Q15 and in the medical report, not here.
Q14 — Steps to find compatible work	Describe what you've tried, and be truthful if the answer is “very little, because of my health.” Frame around unpredictability and sustainability, not preference. Example: “I have looked for work I could do on good days, but my symptoms are too unpredictable to commit to a schedule.”
Q15 — How your disability affects work (marked optional)	Treat this as the most important thing you write. It is your one open space. Use AISH-side language: what you <i>cannot</i> reliably do, how your condition is <i>unpredictable</i> , what trying to work has <i>cost</i> you, and why no arrangement makes it sustainable. Write it in your own voice; change every detail to your truth.
Q16 — Currently employed?	If you do a small amount of work, say Yes and describe it truthfully. A little work, at great cost, is evidence of how hard work is — not proof you're fine. “Doing a little, at great cost” belongs on the AISH side when you frame the cost.
Q21 — Reason for leaving a job	If your health ended the job, check “Medical condition.” Do not soften it to “quit” if the real reason was your disability. “Quit” reads as a choice; “medical condition” reads as the disability being the main factor — the AISH test.

A model for Q15 (edit every bracket to your own truth)

My disability affects my ability to work in the following ways:

Unpredictability: My condition flares without warning. There are [days/weeks] I cannot leave home or function, and I cannot know in advance which days those will be. I cannot promise an employer I will show up.

Dependence on others: I can do tasks when someone is beside me directing me step by step, but on my own I cannot reliably start, organize, or finish what a job needs. Without that constant support I cannot hold the work.

The cost of trying: I have tried to work by [taking pain medication to get through / working from home on good days]. The days I work leave me [unable to function for days after]. The work cost me more than it gave, and I could not keep it up.

Everyday functioning: Day to day I struggle with [meals / appointments / medication / focus beyond X minutes]. These are not rare bad days — this is my ordinary week.

Why this works: every line points at *cannot sustain*, *unpredictable*, and *cost of trying* — the AISH fault lines — without a single “if I had the right support” that would flip it toward ADAP.

5. The medical report (DS2444B): the fields that decide it

The physician's report carries the determination. It is where the referral-to-panel decision is effectively made. The completing professional should document the true clinical picture in the language the test weighs — limitation and permanence, not the accommodations that would let the patient work. Below are the fields where wording is decisive.

Field	What the wording must do
Q7 — Medical history: delineate impairment, duration, restriction	The central narrative field. Tie each condition to a duration and a functional restriction, then close on permanence and impact on earning capacity. Aim for a sentence like: "In my opinion the condition substantially limits this patient's ability to earn a livelihood on a reliable, sustained basis, and is not resolved by workplace accommodation."
Q40 / Q68 — Cumulative ADL and mental-health impairment (free text)	Adjudication weighs cumulative burden across domains. Rate each item to the true picture (0–3) — an accurate 2 or 3 with a one-line note carries the file; do not under-rate to look conservative. Note variability ("3 on flare days, 1 between") rather than averaging it away. Close on: cannot independently maintain the routine and reliability sustained employment requires.
Q69–Q74 — Remedial therapy (the permanence gate)	Denials often turn on "more therapy might help, so it isn't permanent yet." Close that gate where it is clinically closed: treatments tried and exhausted, and why further treatment isn't expected to restore work capacity. If a treatment is nominally available but inaccessible for [12–16+ months], say so — therapy the patient cannot access is not meaningfully available.
Q75 — Prognosis (the determinative field)	Four options: Temporary, Episodic, Permanent, Undetermined. Where clinically supported, choose Permanent. For episodic conditions, choose Episodic and quantify frequency, duration, severity, and unpredictability — unpredictable recurrence is itself a barrier. Avoid "Undetermined" if the record supports permanence: an ambiguous prognosis defaults the file toward ADAP.
Q76 — Additional comments (the synthesis)	The closing statement that draws the domains together: cumulative impairment across [physical/cognitive/mental-health], limitations [continuous/unpredictable], not correctable by accommodation, not expected to improve with treatment — therefore substantially limited in the ability to earn a livelihood on any reliable, sustained basis.

The three phrases that carry a medical report toward AISH

- **“Not correctable by workplace accommodation”** — pre-empts the ADAP “capable with support” finding. Pair it with the reason: the limiting factor is unpredictable incapacity, not the work environment.
- **“On a reliable, sustained basis”** — shifts the question from “can they ever do a task?” to “can they hold a job?”, which is what “earn a livelihood” actually means.
- **“Not expected to improve with treatment” / “options exhausted”** — closes the permanence gate that separates “permanent” (AISH) from “may improve” (ADAP).

6. Special situations worth naming

Transitioned to ADAP on July 2, 2026	On Q1 of the applicant form, check “Yes — my file was transitioned to ADAP as of July 2, 2026.” This frames you as a current client asking to be reassessed for AISH, not a brand-new applicant. Alberta covers the cost of one Disability Medical Report for this reassessment, and it is not time-limited.
Automatic AISH categories	You stay on AISH automatically (unless you chose ADAP) if you have a severe and profound developmental disability or PDD services, a palliative/terminal condition, live in continuing care, or are 60+. Flag any of these clearly at the top of the form — they short-circuit the whole language question.
Episodic conditions	“Episodic” alone does not distinguish the programs — ADAP’s own test says “continuously or episodically.” What tips episodic toward AISH is quantified unpredictability: frequency, length, severity, and the fact that you cannot know which days you’ll be unable to function, so no schedule can be committed to.
Doing a little work	Report it truthfully. Framed with its cost (“the days I work leave me unable to function for days after”), a small amount of work is evidence of how hard work is, not proof you can sustain it. Framed without cost, it reads as capacity — and capacity is ADAP.

7. The guardrail: accuracy is the strongest position

None of this is about overstating. It is about describing a real impairment fully, in the words the determination actually weighs. That distinction matters for a practical reason as well as an ethical one:

- An overstated report can be challenged on review and can cost someone the case they genuinely had. Accuracy is the position that withstands scrutiny.
- The physician’s signature certifies clinical opinion; your signatures certify truth. Every bracket and template should be completed from the real record and the real week.
- When a box is too small for the truth, write “see attached” and add a page. Leaving something out that is found later does more damage than the thing itself.

Put simply: tell the whole truth, and tell it in AISH’s vocabulary where AISH is the truth. If ADAP is the honest picture — significant restriction, but work is genuinely possible — then ADAP’s language is the right language, and ADAP carries strong benefits (among the highest core rates in Canada and the highest earnings exemptions). The goal is the program that matches your real situation, reached without letting hedged wording round you down by accident.

Sources

- [Alberta.ca — Alberta Disability Assistance Program \(ADAP\), incl. combined application & routing](#)
- [Alberta.ca — ADAP eligibility \(medical criteria\)](#)
- [Alberta.ca — AISH eligibility \(medical criteria\)](#)

- [Alberta.ca — AISH program overview](#)
- [Companion source documents: DS2444A \(your part\) and DS2444B \(medical report\) completion guides — The Alberta Disability System Breakdown](#)